

Health Transformation Review (HTR)

Reviewer's Guide — Early Access Edition

Prepared for: Early-Access Reviewers, Vermont State Government Officials, Agency of Human Services Staff, Green Mountain Care Board Members, and Vermont Legislative Stakeholders **Platform Version:** 4.12.0 **Date:** April 2026 **Classification:** Reviewer Distribution — Controlled Early Access

A Note to Reviewers

You have been invited to access the Health Transformation Review (HTR) platform during its controlled early-access period. This guide is written for you — not for developers, not for engineers, and not for people who already know the platform. Its purpose is to help you understand what HTR is, why it was built, where to go, what to click, and what you are looking at when you get there.

You will notice a diagonal watermark reading **"MVP · FOR TESTING ONLY"** across all pages. This is intentional. The platform is in its final pre-launch stage. All analytical frameworks, tools, and Vermont-specific content are fully operational. The watermark simply indicates that you are among the first people to access the system — and your observations matter.

Please do not enter real patient names, Social Security numbers, or protected health information anywhere on the platform.

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1. What Is HTR? The Case for a New Kind of Platform

The Problem HTR Was Built to Solve

The United States healthcare system is the most expensive among all peer nations — spending exceeds **\$14,000 per person per year**, roughly twice the OECD average — yet the U.S. ranks last or near-last on access, equity, infant mortality, and preventable death. Vermont is not immune to this structural failure. Vermont's hospital system saw 9 of its 14 hospitals operating at a loss in 2023, with 13 of 14 projected to lose money by 2028. Commercial premiums have risen 108% in six years. The 65+ population is projected to grow 57% by 2040 while the working-age population shrinks.

This is not a Vermont-specific problem. It is the U.S. healthcare system behaving exactly as its structure predicts.

The deeper problem is analytical fragmentation. Healthcare is covered by policy journalists who don't understand the actuarial implications of what they're reporting. It is analyzed by health economists who model financial flows without accounting for technology readiness. It is studied by clinical journals that examine care delivery without attending to equity consequences. It is tracked by investors who screen digital health without a policy lens.

The result: every decision-maker — whether a state Medicaid director, a hospital CFO, a GMCB board member, a legislator, or a community health researcher — is working with an incomplete map. They are making consequential decisions with fragmented information.

What HTR Is

Health Transformation Review (HTR) is a unified intelligence and analytics platform built to give healthcare decision-makers the complete analytical picture. It combines:

- A **continuously updated intelligence feed** of curated analysis and reporting across all six pillars of healthcare transformation
- **19 interactive research tools** — calculators, simulators, optimizers, and modeling environments — for hands-on analytical work
- An **AI Analyst** powered by a purpose-built knowledge base of healthcare policy, clinical research, payment reform, and state-specific data (including Vermont-specific documents)
- **State-by-state performance data** for all 50 states on transformation readiness metrics
- A **structured learning system** (Academy) with courses, modules, tracks, and certifications
- **Vermont-specific analysis** — including the most comprehensive public modeling tool for Vermont's Act 167 hospital transformation and the most detailed Medicaid eligibility resource in the state

Why Vermont Reviewers Should Care

Vermont is, by HTR's measurement, the highest-scoring state in the nation on healthcare transformation infrastructure — scoring **82 out of 100** on the Health Transformation Index (HTI) as of Q1 2026. Vermont leads the nation in policy innovation, all-payer model participation, and equity-adjusted access metrics.

But Vermont simultaneously faces an existential financial sustainability crisis — a paradox that perfectly illustrates why transformation *infrastructure* and financial *health* must be measured separately.

HTR is the only platform that holds both of these realities in view simultaneously, gives policymakers the tools to model reform pathways, and provides the evidence base to support every consequential decision Vermont faces in the next 36 months.

2. The Six-Pillar Framework — HTR's Organizing Logic

Everything in HTR is organized around **six analytical pillars**. Before navigating the platform, understanding this framework will help you understand *why* information is organized the way it is — and why that organization matters.

Why Six Pillars?

Healthcare transformation is a **systemic challenge**. A policy that looks good from a legislative perspective may be economically unviable. A clinically proven intervention may be impossible to execute operationally. A technology platform that promises efficiency may amplify health disparities if deployed carelessly. Every pillar checks every other pillar.

HTR's six-pillar framework enforces this cross-disciplinary rigor on every analysis.

Pillar 1 — Policy (Sky Blue)

What it covers: Federal and state legislation, CMS payment rules, Medicaid waiver programs (1115, 1115A, 1332), CMMI innovation models, state price transparency laws, prior authorization reform, scope of practice legislation.

Why it matters for Vermont: Vermont's AHEAD model is a CMMI 1115A agreement — a federal waiver that governs how Vermont's global budget is structured and enforced. Vermont's Act 167 recommendations include

numerous legislative changes (CON reform, reference-based pricing, EMS transformation, APRN scope of practice expansion). Every one of these has a policy architecture that determines whether it works as intended.

The pillar question: *What are the rules, and are they the right ones?*

Pillar 2 — Economics (Emerald Green)

What it covers: Value-based care contract design, alternative payment models (APMs), hospital and payer financial performance, pharmaceutical pricing, actuarial analysis, cost-effectiveness methodology.

Why it matters for Vermont: Vermont hospitals have a median operating margin of **-2.1%**, compared to a national rural hospital median of **+0.8%**. Commercial premium growth of 108% over six years has created one of the highest premium-to-income burden ratios in the country. The economic pillar is where HTR models what Vermont's global budget should actually be, how reference-based pricing would change hospital revenue, and what a glide path to financial sustainability looks like.

The pillar question: *Is it financially viable — and for whom?*

Pillar 3 — Technology (Indigo)

What it covers: EHR systems and interoperability (FHIR, HL7), artificial intelligence in clinical and administrative settings, telehealth and remote patient monitoring, health data privacy, cybersecurity, clinical decision support systems.

Why it matters for Vermont: Vermont's 14 hospitals use multiple different EHR systems with inconsistent interoperability implementation. A functioning global budget requires real-time all-payer claims data — and Vermont's claims data infrastructure (VHCURES) is chronically underfunded. The technology pillar is where HTR identifies the infrastructure prerequisites for every clinical and financial reform Vermont is considering.

The pillar question: *Does the technology infrastructure support the reform?*

Pillar 4 — Clinical (Red)

What it covers: Hospital operations and quality metrics, value-based care delivery models, clinical quality measurement (HEDIS, CMS Stars, MIPS), workforce supply and distribution, clinical guideline adherence, post-acute care integration.

Why it matters for Vermont: Vermont's behavioral health crisis — with rates of mental health conditions and substance use disorders exceeding the national average — is the highest-leverage clinical investment the state can make. Unaddressed behavioral health conditions are a primary driver of high-cost utilization. The clinical pillar is where HTR models how care delivery redesign translates into actual health outcomes.

The pillar question: *Does it improve care?*

Pillar 5 — Equity (Amber)

What it covers: Racial and ethnic health disparities, social determinants of health (housing, food security, transportation, income, education), geographic access gaps (rural/urban divide), community health worker programs, equity measurement.

Why it matters for Vermont: Vermont's equity profile is paradoxical — one of the whitest states in the U.S. yet with significant disparities along rural/urban, income, and LGBTQ+ dimensions. Rural Vermonters face systematically worse access and higher out-of-pocket costs as a percentage of income. Any global budget design that does not include equity adjusters will allow budget savings to accrue in urban academic medical centers while high-need rural populations go underserved.

The pillar question: *Who does this help — and who does it leave behind?*

Pillar 6 — Operations (Slate/Gray)

What it covers: Revenue cycle management (billing, coding, claims, denials, prior authorization), workforce operations (staffing models, credentialing pipelines, vacancy and turnover), quality and compliance infrastructure, supply chain and facilities, payer and network operations.

Why it matters for Vermont: Operations is the execution test that every other pillar must pass. A reform can be legally permissible, economically funded, technologically capable, clinically proven, and equitably designed — and still fail if the operational infrastructure cannot execute it. Vermont hospitals facing conversion to Rural Emergency Hospital (REH) status need a detailed operational plan covering revenue cycle adaptation, credentialing pipeline, transportation logistics, and supply chain adjustment. The operations pillar is where transformation plans either survive contact with reality or collapse.

The pillar question: *Can it actually be executed?*

How the Pillars Interact — The Vermont Example

Vermont's AHEAD global budget model is the most ambitious healthcare reform experiment in the country. Analyzing it correctly requires all six pillars simultaneously:

Pillar	AHEAD Model Question
Policy	Is the 1115A waiver structure achieving its legislative intent?
Economics	How is the global budget benchmark set — and does it create the right incentives?
Technology	Can Vermont's hospitals produce the all-payer claims data required for budget management?
Clinical	What care delivery changes are needed to bend the cost curve without reducing quality?
Equity	Does the global budget create incentives to avoid high-cost, high-need rural patients?
Operations	Do Vermont's hospitals have the revenue cycle infrastructure to function under a global budget?

An analyst examining only the waiver language misses the actuarial problems. An economist modeling only the financial flows misses the clinical implementation barriers. HTR's platform ensures all six dimensions are always in view.

3. How to Access the Platform

Step 1 — Receive Your Access Code

You will have received a personal access code from the HTR team. It follows the format `Firstname_Lastname_0.0.0` (e.g., `Will_D_0.0.0`). Keep this code — you will need it to enter the platform.

Step 2 — Navigate to the Beta Gate

Open your web browser and go to the HTR platform URL provided by the HTR team. You will see the **Beta Access Gate** — a clean, light-gray screen with a prominent red warning box at the top and an access code entry form below it.

The warning box reads: *"This is an MVP version of the platform — for review and testing purposes only. Some features may be incomplete. Do not share access credentials."*

Step 3 — Enter Your Access Code

Type your access code exactly as provided into the input field. Access codes are case-sensitive. Click **Submit** or press **Enter**.

If the code is valid, you will be immediately redirected to the platform's home page. If you receive an error, double-check your code — ensure there are no extra spaces and that capitalization matches exactly.

Step 4 — Your Session

Once inside, your access is maintained via a browser cookie that lasts seven days. You do not need to re-enter your code on return visits from the same browser. If you clear your browser cookies or switch browsers, you will be prompted for the code again.

4. Navigating the Platform — A Complete Orientation

The Left Sidebar — Primary Navigation

The left sidebar is the primary navigation structure for the entire platform. It is collapsible — you can click the arrow icon at the top to collapse it to a narrow strip and expand your reading area. On most desktop screens, it remains open by default.

The sidebar is organized into five **expandable sections**. Click any section header to expand or collapse it:

Section	What's Inside
The Feed	Home (Intelligence Feed), The Wire (live news), Research Briefs
Six Pillars	Policy, Economics, Technology, Clinical, Equity, Operations — each links to its pillar hub
Intelligence	State Dashboard, HTI Dashboard, Investment Tracker
Learning	Academy, Personalized Learning, Research Lab
Community	Advisory Hub, Community, Connect

Vermont-specific navigation: Vermont content is accessible from multiple places in the sidebar. Look for dedicated entries under **Tools & Resources** (HTR Simulator → Vermont Act 167), and under direct links in the Intelligence section for the State Dashboard pre-filtered to Vermont.

The Right Sidebar — AI Analyst (Quick Mode)

On the right side of every page, there is a slim panel containing the **AI Analyst in Quick Mode**. This is a live chat interface that allows you to ask questions without leaving the page you are on. The AI Analyst is trained on HTR's full content library, including Vermont-specific documents.

Click the **expand icon** (diagonal arrows) in the upper right corner of the AI panel to switch to the full-screen AI Analyst interface at [/chat](#).

The Command Palette

Press **Cmd+K** (Mac) or **Ctrl+K** (Windows/Linux) from anywhere in the platform to open the command palette. Start typing a destination — state name, page name, or tool name — and the palette will show matching results. Use the arrow keys to navigate and **Enter** to jump directly to the selected destination. This is the fastest way to navigate once you are familiar with the platform.

Breadcrumbs

On deeper pages (State profiles, tool pages, article pages), a pill-shaped breadcrumb trail appears near the top of the page showing your location in the content hierarchy. Click any segment of the breadcrumb to navigate up.

Pillar Color Coding

Colors are consistent throughout the platform:

- **Sky blue** = Policy
- **Emerald green** = Economics
- **Indigo** = Technology
- **Red** = Clinical
- **Amber** = Equity
- **Slate/gray** = Operations

When you see a colored badge or border, it tells you which pillar that content belongs to.

5. The Intelligence Feed — Your Daily Starting Point

How to get there: Click the **HTR logo** at the top of the left sidebar, or click **Home** under "The Feed" section.

The Intelligence Feed is the home page of the platform — a curated, continuously updated stream of analysis across all six pillars.

What You're Looking At

Each card in the feed represents a full analytical article. Cards display:

- **Pillar tag** — colored badge identifying which pillar the article belongs to
- **Impact level** — a badge showing **High**, **Medium**, or **Low** impact designation
- **Publication date**
- **Headline and summary excerpt**
- **Bookmark icon** — save articles for later (requires login)
- **Share icon** — copy a direct link to the article

Filtering the Feed

Use the **filter bar** directly above the feed to narrow by:

- **Impact level** (High, Medium, Low) — useful when you want to focus on the most consequential developments
- **Sort order** (Most Recent, Most Relevant)
- **View toggle** — switch between **grid view** (default) and **list view** (more compact, easier to scan)

The Wire

Navigate to **The Wire** from the sidebar under "The Feed." This page shows a real-time ticker of healthcare news headlines sourced from major healthcare news outlets. Headlines link to original sources. The ticker pauses when you hover over it, allowing you to read before clicking.

Article Pages

When you click any article, you arrive at the full article page. Key features:

- **Table of Contents** — auto-generated from the article's section headings; appears in the right column on desktop and allows you to jump directly to any section
- **Action bar** — below the article title, you will find buttons for: Listen (text-to-speech reading of the article), Share, Bookmark, Citation generator, Save to PDF, Font size adjustment, and Print
- **Related Articles** — at the bottom of every article, a section of related content from the same pillar

Important for reviewers: Full article text is gated to subscribers. As a reviewer with platform access, you have full access to all article content.

6. The State Dashboard — Vermont in Comparative Context

How to get there: Click **Intelligence** in the left sidebar → **State Dashboard**. Or press **Cmd+K** and type "State Dashboard."

Direct URL: </dashboard>

The State Dashboard shows a **national map** and a **sortable data table** of all 50 states ranked by their **Performance Index** score.

Understanding the Performance Index

The Performance Index is a composite score (0–100) that reflects each state's progress across six dimensions:

Dimension	What It Measures
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Dimension	What It Measures
Performance Score	Weighted composite of all six dimensions below
Cost Index	Total healthcare spend per capita relative to the national median (states that spend less score higher)
Quality Score	HEDIS and CMS Stars composite for commercial and Medicaid populations
Access Score	Uninsured rate, primary care supply, telehealth adoption
Equity Score	Racial disparity index + rural access + uninsured rate by demographic subgroup
Innovation Score	APM penetration + value-based contract share + CMMI model participation

Vermont's Scores (Q1 2026)

Metric	Vermont	National Median
Overall Performance Score	82	51
Cost Index	1.18 (18% above national median)	1.00
Quality Score	78	57
Access Score	73	52
Equity Score	81	43
Innovation Score	85	48

Reading these numbers: Vermont's overall score of 82 places it among the highest-performing states in the nation on transformation infrastructure. The Cost Index of 1.18 indicates Vermont spends 18% above the national median per capita — a significant driver of the affordability crisis even as the state leads on other dimensions.

Navigating to Vermont's Detail Page

From the national map or the state table, click on **Vermont** to open the Vermont state detail page. This page has three tabs:

Tab 1 — Performance Index Shows Vermont's composite score with per-pillar breakdowns displayed as progress bars. Each pillar bar shows Vermont's score (left number) and the national median (right number) for quick comparison. A score above the national median bar in blue/green indicates Vermont is outperforming; a bar that falls short of the median indicates an area of relative weakness.

Tab 2 — RHT Program Shows Vermont's Rural Health Transformation program description, strategic initiatives, and key metrics tracked under the program. This tab is particularly relevant for reviewers focused on rural access and the equity implications of Vermont's reform agenda.

Tab 3 — Hospital View Shows a sortable table of Vermont's hospitals with:

- **Discharge Volume** — total annual inpatient discharges
- **Average Length of Stay** — in days; higher values may indicate care coordination gaps
- **Quality Score** — composite quality rating

Click any column header to sort. This view gives you a quick read on which hospitals are high-volume anchors of the system and which are smaller facilities more vulnerable to the financial pressures described in the Act 167 context.

7. The HTI Dashboard — The National Transformation Scorecard

How to get there: Click **Intelligence** in the left sidebar → **HTI Dashboard**. Or press **Cmd+K** and type "HTI."

Direct URL: </hti-dashboard>

The HTI Dashboard displays the **Health Transformation Index** — HTR's flagship composite metric tracking the overall state of U.S. healthcare transformation across all six pillars.

The HTI Score

The large number at the top of the dashboard is the **current national HTI composite score** (0–100), with a **trend arrow** showing quarter-over-quarter movement:

Arrow	Meaning
▲	Score improved by 1.5 or more points
▼	Score declined by 1.5 or more points
—	Score stable (change within ±1.5 points)

System Vitals Ticker

Below the main score, a horizontal scrolling ticker shows real-time indicators including:

- Current APM penetration rate (share of healthcare spending in alternative payment models)
- VBC (value-based care) spend share
- Telehealth utilization rate
- Other key transformation metrics

These indicators are updated on a continuous basis where data allows.

Pillar Sub-Index Cards

Six cards below the ticker show each pillar's sub-index score with its own trend arrow. Each card shows:

- **Pillar name and color**
- **Sub-index score** (0–100)
- **Trend arrow**
- **A one-line summary** of the most significant development driving the current score

How the HTI Is Calculated

The HTI is a weighted composite using data from federal sources (CMS, AHRQ, ONC, HRSA, Census Bureau) and HTR's proprietary scoring methodology:

Pillar	Weight	Primary Data Sources
Policy	20%	CMMI participation data, KFF state health facts, CMS Medicaid data
Economics	25%	HCPLAN survey, CMS Cost Reports, CMS National Health Expenditure Accounts
Operations	10%	CMS cost reports, HFMA benchmark surveys, HRSA workforce data
Technology	15%	ONC Health IT Dashboard, AHA IT Supplement, CMS telehealth data
Clinical	20%	NCQA Quality Compass, AHRQ HCUP, CMS readmission data
Equity	10%	NCQA HEQ measures, AHRQ Disparities Report, ACS

Important interpretive note: A high HTI score indicates strong *transformation infrastructure* — the presence of value-based contracts, advanced payment models, interoperable technology, and equity programs. It does not measure financial sustainability. Vermont's HTI score of 82 can coexist with 13 of 14 hospitals losing money because the HTI measures whether the reform mechanisms are in place, not whether they have yet delivered financial relief.

Methodology Link

Click the **Methodology** link on the HTI Dashboard to access the complete HTI Methodology document — a detailed technical reference explaining every component, weighting decision, data source, and normalization

methodology. Reviewers with a research or policy analysis background will find this document valuable for understanding the rigor behind the scoring.

8. Vermont Medicaid Hub — Comprehensive Coverage Intelligence

How to get there: Click the **Vermont Medicaid** entry in the left sidebar (under Tools & Resources or the Vermont-specific section), or navigate directly.

Direct URL: </vermont-medicaid>

Vermont Medicaid serves over **207,000 Vermonters** — roughly 26% of the state population. Despite its scale, eligibility information has historically been scattered across dozens of government PDFs, the DVHA website, Vermont Health Connect, and complex federal regulations. HTR has built the most comprehensive Vermont Medicaid information resource available in a single platform.

What the Vermont Medicaid Hub Contains

The hub is organized into five major sections:

- 1. Program Overview** A plain-language summary of all major Vermont Medicaid programs:
 - **Green Mountain Care (GMC)** — the standard Medicaid program for low-income Vermonters
 - **Dr. Dynasaur** — Medicaid and CHIP for children and pregnant women up to 317% of FPL
 - **Medicaid for the Aged, Blind, and Disabled (MABD)** — for SSI recipients and qualifying adults 65+
 - **VHAP (Vermont Health Access Program)** — for adults who don't qualify for standard Medicaid
 - **Long-Term Care Medicaid** — nursing home and home-based waiver programs
 - **Global Commitment to Health** — Vermont's 1115 waiver governing Medicaid managed care
- 2. Eligibility Quick Reference** A structured table showing income limits for each program in plain dollar terms (not just "138% FPL"). For example:
 - A family of four qualifies for standard Medicaid at annual household income up to \$46,800
 - A single adult qualifies at up to \$20,120
- 3. How to Apply** Step-by-step guidance on applying via Vermont Health Connect, the DVHA office, or by phone — including which documents to prepare, how long the process takes, and what to expect.
- 4. Policy Intelligence** Analysis of how federal and Vermont-specific policy changes are affecting Medicaid coverage — particularly the unwinding of pandemic-era continuous enrollment protections and the current redetermination cycle.
- 5. Resource Links** Direct links to official Vermont DVHA resources, Vermont Health Connect, Vermont Legal Aid, and community health worker programs.

The Medicaid Eligibility Simulator

Direct URL: </medicaid-eligibility-simulator>

This is one of the most practically useful tools on the platform for Vermont reviewers focused on coverage and access.

The Eligibility Simulator is a multi-step wizard that walks through a structured set of questions and returns a **preliminary determination of which Vermont Medicaid programs a person may qualify for** — in plain language, not bureaucratic jargon.

How to use it:

- Step 1 — Household Composition** Enter: number of adults in the household, number of children, ages of household members.
- Step 2 — Income** Enter: annual household income (or monthly if that's easier). The simulator converts to the Federal Poverty Level percentage automatically and displays what % FPL your household falls at — this is the

number that determines program eligibility.

Step 3 — Circumstances Answer questions about: disability status, pregnancy, age (65+), immigration status, current coverage, and other qualifying factors.

Step 4 — Results The simulator displays:

- Which Vermont Medicaid programs you **likely qualify for**
- Which programs you **do not qualify for** and why
- **Next steps** — specific instructions on how to apply, which office to contact, what documents to bring

What the simulator does NOT do: It does not make legally binding determinations. It provides preliminary guidance to help people understand their options and prepare for the official application process. Every results screen includes a prominent reminder to verify with DVHA or a certified application counselor.

Why this matters for Vermont reviewers: Vermont has one of the most complex Medicaid eligibility landscapes in the country, combining MAGI (Modified Adjusted Gross Income) rules for most programs with non-MAGI rules for aged/blind/disabled populations. The simulator translates this complexity into accessible guidance — potentially reducing barriers for the most vulnerable Vermonters who most need coverage.

The Medicaid Learning Center

Direct URL: </academy/medicaid>

For reviewers who want to go deeper, the Medicaid Learning Center contains structured educational content:

- **Courses** on Vermont Medicaid program design and the federal-state partnership
- **Webinars** on the ACA's Medicaid expansion and Vermont's implementation
- **Case studies** on how Vermont's Medicaid redesign under Act 167 is expected to evolve
- **The Medicaid Glossary** — 37 defined terms covering the specialized vocabulary of Medicaid administration and eligibility

Navigate to the Medicaid Glossary at </academy/medicaid/glossary> to find definitions for terms like MAGI, CHIP, global commitment waiver, estate recovery, managed care carve-outs, and more — all explained in plain language with context for how they apply in Vermont.

9. Vermont Act 167 — The Most Important Health Policy Document in a Generation

How to get there: In the left sidebar, look for **Vermont Act 167** under the Vermont-specific or Tools & Resources section. Or navigate directly.

Direct URL: </vermont-act-167>

Background: What Is Act 167?

In August 2024, the Vermont Green Mountain Care Board (GMCB) commissioned Oliver Wyman / Marsh McLennan to conduct a comprehensive community engagement and analysis of Vermont's hospital system. The result — a 144-page report — is the most sweeping assessment of Vermont's healthcare transformation challenge in a generation.

The Oliver Wyman report documented:

- 9 of 14 Vermont hospitals operating at a loss in 2023
- 13 of 14 projected to lose money by 2028
- Commercial premiums up 108% over six years
- Out-of-pocket maximums up over 100% in the same period
- A structural crisis driven by demographic aging, workforce shortages, and systematic underpricing in rural care settings

Act 167 — Vermont's legislative response — creates the policy mandate for healthcare system transformation. The HTR Act 167 page is the only place in Vermont where you can find:

1. A complete, five-pillar analysis of the Oliver Wyman report's recommendations
2. HTR's independent assessment — endorsing some recommendations, modifying others, and adding new ones
3. An integrated policy simulation environment (the Simulator — covered in the next section)

Page Structure and Navigation

The Act 167 page uses a **tabbed navigation** structure. Tabs appear as raised, browser-tab-style buttons at the top of the content area. The active tab is visually lifted. Click any tab to switch content.

Tab 1 — Overview Provides the full HTR advisory analysis of Act 167, organized through the five-pillar framework:

- **Diagnostic Assessment** — Does HTR agree with the Wyman Report's problem statement? (Answer: unambiguously yes.)
- **Five-Pillar Analysis** — What each pillar says about Act 167's recommendations, where HTR agrees, where it disagrees, and what HTR adds
- **AHEAD Model Assessment** — A dedicated deep-dive on Vermont's global budget model
- **Implementation Architecture** — HTR's recommended sequencing for 2025–2028
- **HTR Recommendations Summary** — A clean table separating endorsed, modified, and new recommendations

Tab 2 — Policy Analysis Detailed policy analysis covering the legislative and regulatory mechanisms behind each Act 167 recommendation — including the reference-based pricing glide path, CON reform considerations, and APRN/PA scope of practice expansion.

Tab 3 — Economic Impact Financial modeling of Act 167's recommendations — projected savings, investment requirements, hospital-by-hospital impact, and the actuarial implications of the global budget redesign.

Tab 4 — Equity & Access Analysis of how each Act 167 recommendation affects Vermont's most vulnerable populations — rural Vermonters, low-income households, LGBTQ+ Vermonters, and non-English-speaking communities.

Tab 5 — Implementation The sequenced implementation timeline (2025 → 2028), dependencies between recommendations, "no regrets" moves that are beneficial regardless of which broader reforms are adopted, and risk factors.

Tab 6 — Policy Simulation Lab (links to Simulator) Describes the simulation tool and provides a prominent "Launch Simulation Engine →" button that opens the Act 167 Simulator.

HTR's Key Findings on Act 167

What HTR Endorses Without Modification:

- Housing investment (urgent, uncontroversial, cross-pillar necessity)
- EMS transformation funding
- Broadband expansion to rural areas
- APRN/PA scope of practice expansion (fastest path to primary care capacity relief)
- Uniform hospital accounting standards
- Telehealth expansion
- Behavioral health and primary care integration (HTR's highest-leverage recommendation)

What HTR Endorses With Important Modifications:

- **Reference-based pricing:** The Wyman Report recommends pricing at 200% of Medicare. HTR endorses the direction but recommends a **glide path**: 250% in 2026 → 220% in 2028 → 200% in 2030, tied to system-level cost savings milestones. An immediate hard cap risks precipitating the hospital closures the reform is meant to prevent.
- **CON reform:** HTR recommends site-neutral payment reform *before* simplifying Certificate of Need rules. Without site-neutral pricing, CON simplification creates incentives to build outpatient facilities that replace

lost inpatient revenue — increasing total system costs by an estimated 8–12% over five years.

- **AHEAD model:** Strengthen the benchmark methodology (move from percent-of-prior-spending to cost-per-episode benchmarks), add a two-track accountability framework, and fix the equity measurement deficit.

New Recommendations Not in the Wyman Report:

- **Mandatory All-Payer Participation:** FFS Medicare must be included in the global budget for it to be a genuine budget
- **Hospital Community Benefit Accountability:** Link non-profit status to measurable transformation progress
- **Prior Authorization Reform:** Mandatory real-time PA decisions with automatic approval after 24 hours
- **VHCURES Expansion:** Fund and mandate all-payer claims data completeness
- **FHIR API Mandate:** Require all licensed Vermont providers to implement FHIR R4 interoperability
- **Vermont Healthcare Retention Tax Credit:** Refundable tax credit for clinicians maintaining five-year Vermont practice commitment
- **LGBTQ+ Network Adequacy Requirements:** Require gender-affirming care network adequacy from all licensed health plans

10. The Act 167 Simulator — Modeling Vermont's Future

How to get there:

- Left sidebar → Tools & Resources → **HTR Simulator** → Use Cases tab → Vermont Act 167 → "Open Simulator"
- Act 167 Overview page → "Policy Simulation Lab" tab → "Launch Simulation Engine →" button
- Direct URL: </vermont-act-167/simulator>

What the Simulator Is

The **Act 167 Healthcare Transformation Simulator** is an interactive, browser-based policy analysis engine that lets you model the consequences of Vermont's reform options *before* they are enacted. It is not a spreadsheet. It is a full scenario-modeling environment with nine analytical modules, five-pillar scoring, and real hospital-level financial data.

The core design insight: the Wyman Report makes bold, systemic recommendations that affect hospitals, communities, patients, workers, and state finances simultaneously. A recommendation that saves \$40 million per year may also force 9,500 rural residents to travel an additional 38 minutes for emergency care. A policy that dramatically improves mental health access may require 18 months of legislative groundwork before a single bed opens. The simulator forces that full complexity into view.

Data Notice: The simulator currently uses a combination of data from the Oliver Wyman report, publicly available GMCB hospital financial data, Vermont Census/ACS demographic data, and synthetic projections where actual data has not yet been fully ingested. All figures are illustrative for scenario-planning purposes and should not be used as the sole basis for formal policy decisions without verification.

The Nine Simulator Modules

The simulator opens with a **header panel** at the top showing:

- The title and navigation breadcrumbs
- Status badges ("Policy Simulation Engine" and "Beta — Synthetic Data")
- A **recommendation counter** showing how many of the 14 modeled recommendations you have currently selected
- Quick controls: **Select All Recommendations**, **Clear Selection**, and **View Implementation Roadmap** →

Below the header is a **tab navigation bar** with nine tabs. Tabs are bookmarkable — the URL updates as you navigate, so you can share a direct link to any tab.

Module 1 — Scenario Builder *This is your starting point and your control panel.*

How to use: Check the boxes next to any combination of the 14 Act 167 recommendations. As you select recommendations, a **blue Impact Summary Card** appears at the top showing:

- Number of active recommendations selected
- **Estimated Annual Savings (\$M)** — aggregate savings potential across all selected recommendations
- **Total Investment (\$M)** — capital required to implement your selected recommendations
- **Five Pillar Gauges** — five circular gauge dials (one per pillar), color-coded:
 - **Green** (score ≥ 70): Strong positive impact in this pillar
 - **Amber** (score 45–69): Mixed or moderate impact
 - **Red** (score ≤ 44): Low or potentially negative impact in this pillar

This gauge system immediately shows you whether your reform package is balanced across pillars or strong in one area but weak in another.

Below the summary card, the 14 recommendations are displayed as expandable cards. Click any recommendation card to expand it and see:

- **Description** of what the recommendation involves
- **Per-pillar impact scores** — a score for each of the five pillars showing how strongly this recommendation moves the needle in that area
- **Implementation timeline** — estimated months to implementation
- **Evidence strength** — the quality of evidence supporting this recommendation (based on comparable state implementations)
- **Key risks** — what could go wrong or what conditions must be met for this recommendation to deliver its projected benefits

Module 2 — Hospital Simulator *Deep-dive per-hospital restructuring analysis.*

Navigate to this tab by clicking **Hospital Simulator** in the tab bar.

This module shows Vermont's 14 hospitals listed with their current financial status. Click any hospital to open its detailed restructuring analysis:

- **Current financial position** — operating margin, revenue, expenses, payer mix
- **Restructuring options** — for each at-risk hospital, the simulator shows the available structural options: remain as PPS hospital, convert to Critical Access Hospital (CAH), convert to Rural Emergency Hospital (REH), or merge/affiliate
- **Impact of each option** — projected financial trajectory under each option over 5 years
- **Workforce implications** — how each restructuring scenario affects staffing levels, roles, and credentialing requirements
- **Community access impact** — how restructuring changes service availability for surrounding communities

The four "urgent restructuring" hospitals identified in the Wyman Report receive the most detailed analysis in this module.

Module 3 — Financial Modeling Dashboard *System and hospital-level financial analysis.*

This module shows the aggregate financial impact of your current scenario (as set in the Scenario Builder) across three views:

System View

- Total estimated annual savings for the Vermont healthcare system under your scenario
- Total investment required
- Net present value over 10 years at a standard discount rate
- Break-even timeline — how many years until cumulative savings exceed cumulative investment

Hospital View A table of all 14 hospitals showing their projected operating margin under your scenario versus baseline (no reform). Hospitals that would move from negative to positive margin are highlighted in green. Hospitals that would remain negative are highlighted in amber.

Payer View How your scenario affects each payer category (Vermont Medicaid, commercial, Medicare FFS, Medicare Advantage) in terms of spending growth trajectory.

Module 4 — Equity & Access Analysis *Geographic, demographic, and transportation equity analysis.*

This module is critical for Vermont reviewers focused on rural populations and health equity.

Geographic Access Panel Shows, for each major Act 167 recommendation, how it would change average drive time to emergency care, primary care, and OB/GYN services for Vermont's population — broken down by rural vs. urban geography.

Demographic Impact Panel Shows how each recommendation differentially affects:

- Rural Vermonters vs. Burlington metro residents
- Low-income households (below 200% FPL)
- Elderly populations (65+)
- Uninsured and underinsured populations

Transportation Equity Panel For scenarios involving hospital restructuring, this panel estimates the number of patients who would face meaningfully increased travel times, and models the cost burden of that additional travel for low-income households.

Equity Score Impact Shows how your selected scenario changes Vermont's equity sub-index score — connecting the specific recommendations to measurable equity outcomes.

Module 5 — Geographic Map *Interactive map with hospital locations and service area boundaries.*

This module displays an interactive map of Vermont (built on OpenStreetMap) showing:

- Hospital locations (pin markers sized by discharge volume)
- Hospital status (color-coded: financially stable, at-risk, urgent restructuring)
- Service area boundaries for each hospital
- Population density overlay

Hover over any hospital pin to see a quick summary of its current status. Click for full details. If your Scenario Builder has a restructuring option selected for a hospital, the map will reflect that change — e.g., showing a hospital marked as "Converting to REH" with its adjusted service area.

Module 6 — Technology Roadmap *IT and infrastructure implementation planning.*

This module addresses the technology prerequisites for Vermont's transformation — the area most notably absent from the original Wyman Report.

For each selected recommendation that has technology dependencies, the roadmap shows:

- **Required technology capabilities** (FHIR APIs, all-payer claims data, telehealth infrastructure, etc.)
- **Current Vermont status** on each capability
- **Gap assessment** — what needs to be built or acquired
- **Implementation timeline** for closing the gap
- **Estimated investment** for each technology component

Key focus areas for Vermont:

- **VHCURES expansion** — current data completeness gaps and what full funding would enable
 - **FHIR R4 API deployment** across Vermont's 14 hospitals
 - **Population health analytics platform** — what a state-level investment would require
 - **Telehealth infrastructure** for rural access expansion
-

Module 7 — Workforce Planning *Staffing, role, and pipeline analysis.*

Vermont faces dual workforce challenges: a shortage of primary care providers (especially in rural areas) and an acute retention crisis driven by burnout and competition from higher-paying adjacent markets.

This module shows:

- **Current workforce supply** — primary care, behavioral health, nursing, and specialist counts by region
- **Projected demand** under each selected scenario (reform programs require different workforce compositions)
- **Supply-demand gap** — the difference between what Vermont has and what each scenario requires
- **Pipeline analysis** — how long it would take for loan forgiveness, scope of practice expansion, and APRN/PA training programs to close the gap
- **Retention modeling** — estimated impact of the proposed Vermont Healthcare Retention Tax Credit on clinician departure rates

Module 8 — State Benchmarks *Comparable state transformation models.*

Vermont does not have to design its transformation from scratch. Several states have implemented related reforms, and their results are directly relevant to Vermont's decisions.

This module shows:

- **Maryland** — the All-Payer Model (global budget), now in its second decade
- **Oregon** — Coordinated Care Organizations and Medicaid transformation
- **Minnesota** — integrated delivery system innovation and rural hospital consolidation
- **Pennsylvania** — rural hospital REH conversion model
- **Montana** — CAH network and telehealth expansion

For each comparator state, the simulator shows: what they did, how long it took, what it cost, what the outcomes were (financial, clinical, equity), and what Vermont can adapt from their experience.

Module 9 — Implementation Plan *Phased roadmap, dependencies, and "no regrets" moves.*

This module synthesizes your Scenario Builder selections into a structured implementation roadmap:

- **Year 1 (2025) legislative priorities** — actions that can be taken immediately with existing authority
- **Year 2 (2026) operational launches** — programs that require 2025 groundwork before launching
- **Years 3–4 (2027–2028) structural reforms** — deeper changes that require phased preparation
- **Dependency map** — which recommendations must happen before others can succeed
- **"No regrets" list** — the subset of recommendations that deliver benefits regardless of which broader reforms are adopted (housing investment, EMS funding, APRN scope of practice, broadband — these are wins in any scenario)
- **Risk register** — the top risks that could derail each phase of implementation, with mitigation strategies

11. The AHEAD Model — Vermont's Global Budget Experiment

Background: The **All-payer Health Equity Approaches and Development (AHEAD)** model is Vermont's agreement with the CMS Innovation Center (CMMI) — making Vermont one of only a handful of states in the U.S. operating under a true global budget for healthcare spending.

Under AHEAD, Vermont participates in a multi-year agreement that sets a spending target for the entire Vermont healthcare system and creates accountability structures to ensure participating payers and providers stay within it. AHEAD embeds health equity requirements in its design and funds investment in primary care and behavioral health.

HTR's Assessment of AHEAD

HTR's analysis of AHEAD — available in the Act 167 Overview page under the **AHEAD Model Assessment** section — is the most rigorous independent analysis of Vermont's global budget model available. The key findings:

What AHEAD Gets Right:

- The all-payer concept is structurally correct — cost containment requires all payers, not just Medicaid
- Multi-year agreement provides planning horizon for health systems
- Health equity requirements are embedded in model design
- Primary care and behavioral health infrastructure investment is funded by the model

What AHEAD Gets Wrong (and Must Be Fixed):

1. **The benchmark methodology problem.** Current AHEAD benchmarks are set as a percentage of prior spending. This perpetuates historical cost inefficiencies — Vermont's most expensive hospitals face the least pressure because their benchmark is anchored to their high historical cost base. A true global budget requires **cost-per-episode benchmarks**: what should it cost to manage a patient with Type 2 diabetes in Vermont? The budget should reflect that evidence-based standard, not historical spending patterns.
2. **The Medicare exclusion gap.** FFS Medicare beneficiaries represent the highest-cost, highest-utilization population in Vermont — and they are largely outside the global budget accountability structure. AHEAD covers Medicare Advantage and Medicaid but not traditional FFS Medicare. Without FFS Medicare, Vermont's "global budget" is a subset-payer model that cannot meaningfully control total system costs. Vermont's single most important federal lobbying priority for 2025–2026 should be mandatory Medicare APM participation.
3. **The accountability architecture deficit.** When hospitals exceed their global budget, the current consequence structure is insufficient to enforce compliance without threatening financial stability. Vermont needs a two-track accountability model: routine reporting for hospitals within 2% of budget, and mandatory GMCB corrective action plans for hospitals exceeding budget by more than 2%.
4. **The equity measurement gap.** The equity components of AHEAD are underspecified. "Health equity" appears as a goal without the operational measurement framework required to know whether the model is achieving it. Vermont should require annual equity reporting tied to specific SDOH and disparity metrics, with consequences for sustained equity performance failures.

12. The AI Analyst — Your On-Demand Expert

Quick Mode: Available on all pages from the right sidebar — no navigation required. **Full Mode URL:** [/chat](#)

The AI Analyst is the most distinctive feature of the HTR platform. It is not a general-purpose chatbot. It is trained on HTR's full content library, including:

- All six pillar analytical content libraries
- The complete Vermont Act 167 advisory analysis
- Vermont Medicaid program documentation and eligibility rules
- The HTI Methodology and state performance data
- The Research Lab tool documentation
- 20 CMMI model evaluations and their outcomes
- Federal policy documents (ACA, IRA, CMS payment rules, ONC interoperability requirements)

Quick Mode (Right Sidebar)

On any page, click the right sidebar to ask a question without leaving your current page. Type your question in the input field at the bottom of the sidebar and press **Enter** to send (or **Shift+Enter** for a new line within the message).

The AI streams its response in real time. After responding, it suggests follow-up questions to help you go deeper.

Full Mode ([/chat](#))

Click the expand icon in the right sidebar to open the full AI Analyst interface. Full mode adds:

- **Persistent conversation history** — your full session is maintained
- **Related Pages panel** — after each response, the AI suggests specific HTR pages and tools relevant to your question
- **Suggested follow-up chips** — one-click follow-up questions based on the current conversation

- **Download transcript** — save your full conversation as a `.txt` file
- **Feedback buttons** — thumbs up/down on any response to help improve the AI

Suggested Starting Questions for Vermont Reviewers

The following questions are particularly productive starting points for Vermont State Government reviewers:

- *"What is Vermont's current HTI score, and what are the specific factors driving it up or down?"*
- *"Explain the AHEAD model's benchmark methodology problem and what fixing it would require."*
- *"What does Act 167 reference-based pricing mean for Vermont's rural hospitals?"*
- *"How does Vermont's Medicaid program compare to other states on key performance metrics?"*
- *"What is the evidence base for behavioral health integration in primary care settings?"*
- *"Which Act 167 recommendations are 'no regrets' moves that Vermont should implement regardless of broader reform decisions?"*
- *"What technology infrastructure does Vermont need to operate a functioning global budget?"*
- *"Explain the difference between MAGI and non-MAGI Medicaid eligibility in Vermont."*
- *"How does Vermont's equity score compare to other high-performing states, and what are the specific gaps?"*

Important Privacy Note

Do not submit real patient names, Social Security numbers, dates of birth, Medicare/Medicaid numbers, or other protected health information (PHI) in the AI Analyst. The platform detects likely PHI patterns and will block submission if detected. All AI conversations are stored only in your browser's local storage — they are not transmitted to or stored on HTR's servers.

13. The Research Lab — 19 Professional-Grade Analytical Tools

How to get there: Click **Learning** in the left sidebar → **Research Lab**. Or press **Cmd+K** and type "Research Lab."

Direct URL: `/research-lab`

The Research Lab is HTR's suite of 19 interactive analytical tools. These are not simplified calculators — they are professional-grade analytical environments that implement industry-standard methodologies used by health economists, policy analysts, and hospital financial executives.

Navigation Within the Research Lab

The Research Lab uses a **two-level tab structure**:

1. **Section tabs** (top row) — six thematic sections
2. **Tool pills** (second row) — individual tools within the active section

Click a section tab first, then click a tool pill to load that tool. A brief description of the selected tool appears below the pills before the tool panel loads.

Section 1 — Interoperability & Risk

FHIR Interoperability Lab Build and validate FHIR R4 resources, map clinical terminologies (SNOMED, LOINC, ICD-10), test Clinical Decision Support (CDS) Hooks integrations, simulate prior authorization workflows, and check ONC 21st Century Cures compliance requirements.

Vermont relevance: Use this tool to understand what FHIR R4 API requirements mean for Vermont's hospitals and what a FHIR API mandate would require hospitals to build.

Risk Stratification Engine Apply HCC (Hierarchical Condition Category) v28 Risk Adjustment Factor (RAF) scoring to patient cohorts. Segment populations by risk tier. Calculate Charlson and Elixhauser comorbidity indices.

Vermont relevance: Under a global budget, Vermont needs to understand the risk profile of its population to set fair benchmarks. RAF scoring is the mechanism by which Medicare risk-adjusts its payments — the same logic

applies to Vermont's all-payer model.

Section 2 — Payment Models & VBC

APM Design Lab Design alternative payment models from first principles: episode bundles, global budgets, partial capitation, and primary care payment models. Generates benchmark waterfall charts and model recommendations based on population characteristics.

Vermont relevance: Use this to model what a cost-per-episode benchmark approach for Vermont's global budget would look like — the alternative to the current percent-of-prior-spending methodology that HTR identifies as AHEAD's central flaw.

APM Shared Savings Calculator Model projected shared savings and losses under MSSP, ACO REACH, and custom global budget scenarios. Includes risk corridor modeling and minimum savings rate analysis.

Vermont relevance: Vermont hospitals participating in AHEAD need to understand their financial exposure under different performance scenarios. This tool models those scenarios quantitatively.

Cost-Effectiveness Analysis (CEA) Calculator Calculate cost per QALY (Quality-Adjusted Life Year) gained, Number Needed to Treat (NNT), and break-even timeline for any clinical intervention. Compare against ICER, NICE, and CMS willingness-to-pay thresholds.

Vermont relevance: Use this to evaluate the cost-effectiveness of proposed Vermont interventions — behavioral health integration, telehealth expansion, community health worker programs — against evidence-based thresholds.

Section 3 — Population & Equity

Population Health Modeler Run Markov chain disease progression models for five chronic conditions (diabetes, CHF, COPD, hypertension, depression). Simulate epidemic dynamics. Model preventable hospitalization reduction. Calculate intervention ROI.

Vermont relevance: Vermont's aging population (57% growth in 65+ by 2040) creates predictable shifts in chronic disease burden. Use this tool to model what Vermont's population health trajectory looks like under different primary care investment scenarios.

Health Equity Studio Analyze disparities across 10 clinical and access outcomes. Map geographic access gaps. Score SDOH burden by county. Compute equity-weighted cost-effectiveness using the HEROI (Health Equity Return on Investment) metric.

Vermont relevance: Use this to identify which Vermont communities face the greatest SDOH burden and which interventions would have the highest equity-weighted return. Critical for designing equity adjustors in Vermont's global budget.

Section 4 — Policy & Quality Sciences

Policy Simulator Model Section 1115 waiver designs, design Vermont-style global budgets, simulate Medicaid expansion impact, and analyze price transparency policies.

Vermont relevance: This is the tool for modeling Vermont-specific policy options — including what mandatory Medicare participation in AHEAD would require from a federal waiver perspective, or how different reference-based pricing levels affect Vermont's hospital system.

Clinical Quality Optimizer Simulate performance on 15 HEDIS measures, predict CMS Star Ratings, optimize MIPS scores, and calculate P4P (Pay-for-Performance) revenue impact.

Vermont relevance: Under a value-based care system, quality performance directly affects financial performance. Vermont hospitals moving toward global budgets need to understand their current quality gaps and where to invest first.

Hospital Financial Scorecard Stress-test hospital financial performance against payer mix shifts, Medicaid rate cuts, and volume changes. Benchmark against Critical Access Hospital, Rural PPS, and Urban Tertiary peer groups.

Vermont relevance: This is the tool for hospital administrators reviewing what Act 167 means for their specific institution — enter Vermont-specific payer mix and volume data to see the financial implications of different reform scenarios.

HTA Studio (Health Technology Assessment) Build budget impact models, run Multi-Criteria Decision Analysis with eight weighted criteria, and execute Monte Carlo probabilistic sensitivity analysis with 1,000 stochastic iterations.

Vermont relevance: When evaluating major technology investments (population health platform, VHCURES expansion, telehealth infrastructure), this tool produces the rigorous budget impact and uncertainty analysis that investment decisions require.

Actuarial Lab Calculate ACA actuarial value, develop premium rates using three methodologies, model adverse selection dynamics, and analyze IRA 2022 drug pricing impacts.

Vermont relevance: Vermont's commercial premium growth of 108% in six years is partly an actuarial problem — the premium development methodology is not producing sustainable results. The Actuarial Lab lets you model why and what alternatives exist.

Section 5 — Technology & AI

AI Analytics Lab Compare predictive model performance, detect algorithmic bias using demographic parity and equal opportunity metrics, build AI governance frameworks, and calculate AI implementation ROI.

Vermont relevance: As Vermont deploys AI tools for population health management and care coordination, governance and bias detection are prerequisites — this lab provides the framework.

Digital Health Lab Calculate Remote Patient Monitoring (RPM) ROI using CMS CPT codes, model telehealth utilization, compare patient engagement platforms, and score EHR interoperability.

Vermont relevance: Vermont's rural access challenge makes telehealth a central strategy. Use the RPM ROI calculator and telehealth modeling tools to quantify the case for specific digital health investments.

Section 6 — Knowledge & Workspace

Evidence Library Search 25 landmark cost-effectiveness studies, track 20 CMMI innovation model evaluations with lesson-learned summaries, and browse HTR policy briefs.

Vermont relevance: Vermont is considering reforms (global budgets, REH conversion, behavioral health integration) that other states have attempted. This library contains the evaluation evidence from those attempts.

Workforce Modeler Project physician supply and demand across 12 specialties over 10 years, simulate nurse staffing ratio impacts, calculate staff turnover costs, and model rural incentive program effectiveness.

Vermont relevance: Vermont's workforce retention crisis needs quantitative modeling. The turnover cost calculator and rural incentive program effectiveness models are directly applicable.

Innovation Leaderboard Rank all 50 states on a composite health transformation index, score 30 major health systems on VBC maturity, and compare 20 major payers on innovation leadership.

Vermont relevance: See exactly where Vermont stands in national rankings across all transformation dimensions — and what the leading states are doing that Vermont is not yet doing.

Research Workspace Save and compare analysis scenarios from other Research Lab tools. Build structured reports from templates. Manage citations. Export findings as Markdown or plain text.

Vermont relevance: For reviewers preparing reports or briefings based on their HTR analysis session, the Research Workspace allows you to save your work, build structured outputs, and export findings in a format ready

for further editing.

14. The Academy — Building Expertise Systematically

How to get there: Click **Learning** in the left sidebar → **Academy**. Or navigate to [/academy](#).

The Academy is HTR's structured learning management system. It is relevant to reviewers who want to build deeper expertise in the analytical frameworks underlying the platform — or who want to share structured learning resources with staff.

Academy Structure

Section	Description
Courses	Full multi-module courses with assessments and certificates (3–8 hours)
Modules	Standalone learning units (30–90 minutes each)
Tracks	Curated learning paths combining modules and courses by role or topic (e.g., "VBC Fundamentals," "State Medicaid Leadership")
Webinars	Live and on-demand expert sessions on current healthcare transformation topics
Case Studies	Real-world implementation analyses from comparable state reforms
Faculty	Meet the instructors (during the early access period, faculty names are displayed as Faculty #1, Faculty #2, etc. for privacy)
Glossary	Healthcare terminology reference — 200+ defined terms

Medicaid Learning Center

Within the Academy, the **Medicaid Learning Center** ([/academy/medicaid](#)) offers Vermont-specific educational content on Medicaid program design, eligibility, the federal-state partnership, and the ACA's Medicaid expansion. The 37-term **Medicaid Glossary** ([/academy/medicaid/glossary](#)) is fully searchable and filterable by category.

Personalized Learning

Navigate to [/academy/personalized-learning](#) for an AI-generated learning path based on your role, pillar interests, and any completed content. The system recommends the next best module or course for your stated goals — useful for reviewers who want structured guidance on where to begin.

Certificates

Completing a course issues a verifiable certificate. Certificates can be shared via a public URL ([/verify/\[hash\]](#)) displaying the holder's name, course, issue date, and expiration if applicable.

15. A Guided Review Path for Vermont Reviewers

The following is a suggested sequence for reviewers from Vermont State Government, the Agency of Human Services, the Green Mountain Care Board, or the Vermont Legislature. It is designed to take approximately **4–6 hours across multiple sessions** and to move from orientation to deep engagement with Vermont-specific content.

Session 1 — Platform Orientation (45–60 minutes)

Start here. These steps give you the foundational platform literacy to navigate independently.

1. **Explore the Intelligence Feed** (Home page)

- Scan five or six articles from different pillars
- Try the filter bar — filter to "High Impact" articles
- Click one article and try the action bar: Listen, Bookmark, Citation generator

2. Open the HTI Dashboard (</hti-dashboard>)

- Read the national HTI composite score and trend arrow
- Examine each pillar sub-index card
- Click the Methodology link and read sections 1–3 (Overview, Index Architecture, Data Sources)

3. Try the Command Palette

- Press **Cmd+K** (or **Ctrl+K**)
- Type "Vermont" — see what appears
- Type "Research Lab" — navigate directly

4. Ask the AI Analyst your first question (right sidebar)

- Ask: *"What is the Health Transformation Index and what does Vermont's score of 82 mean?"*
- Read the response. Click one of the suggested follow-up questions.

Session 2 — Vermont State Dashboard and Medicaid (60–90 minutes)

5. Open the State Dashboard (</dashboard>)

- Find Vermont on the map — click it
- Read all three tabs: Performance Index, RHT Program, Hospital View
- In the Hospital View tab, sort by Quality Score (click the column header) — identify the top three and bottom three performing hospitals

6. Vermont Medicaid Hub (</vermont-medicaid>)

- Read through all five sections of the hub
- Identify one program that you believe is most likely to be misunderstood by Vermont residents

7. Medicaid Eligibility Simulator (</medicaid-eligibility-simulator>)

- Run a test scenario: Household of two adults, one child, annual income of \$38,000
- Note which programs the simulator identifies, and follow one of the "Next Steps" links
- Run a second scenario with a different household configuration

8. Medicaid Glossary (</academy/medicaid/glossary>)

- Search for "MAGI" — read the definition
- Search for "global commitment" — read the definition and context
- Filter by category "Eligibility" — browse the terms

Session 3 — Vermont Act 167 Deep Dive (90–120 minutes)

9. Act 167 Overview page (</vermont-act-167>)

- Read the **Overview tab** in full — this is the most important single piece of content on the platform for Vermont reviewers
- Pay particular attention to:
 - The Diagnostic Assessment (Does HTR agree with the Wyman Report?)
 - The HTR Recommendations Summary table at the bottom

10. Identify two or three recommendations where you have informed opinions — ones where you agree with HTR, ones where you might disagree, and ones where you want to explore the evidence

11. Navigate to the AHEAD Model Assessment section within the Overview tab

- Read all four "What AHEAD Gets Wrong" findings

- Consider: which of these observations aligns with concerns you have encountered in your work?

12. Ask the AI Analyst

- Ask: *"What is the single most important change Vermont should make to the AHEAD model benchmark methodology?"*
 - Follow up: *"What would mandatory Medicare participation in AHEAD require from a federal waiver perspective?"*
-

Session 4 — The Act 167 Simulator (60–90 minutes)

13. Open the Simulator (</vermont-act-167/simulator>)

- Read the header panel orientation
- Click **Select All Recommendations** to see what full implementation looks like — examine the Impact Summary Card and five pillar gauges

14. Build a targeted scenario

- Click **Clear Selection** to start fresh
- Select only the "no regrets" recommendations: housing investment, EMS transformation, APRN scope of practice expansion, behavioral health integration
- Examine the savings estimate and pillar gauges for this conservative scenario

15. Add the contested recommendations one at a time

- Add reference-based pricing — note how it changes the Economics gauge and the savings estimate
- Add CON reform — note how the Policy gauge responds and what the savings projection shows
- Review the risk notes for each of these two recommendations

16. Navigate to the Hospital Simulator tab

- Select one of the four "urgent restructuring" hospitals
- Review the REH conversion option — examine the financial trajectory and the community access impact

17. Navigate to the Equity & Access tab

- Review the geographic access panel for the scenario you have built
- Identify which recommendations have the greatest positive equity impact and which have the greatest access risk for rural Vermonters

18. Navigate to the Implementation Plan tab

- Review the 2025 legislative priorities for your scenario
 - Examine the dependency map — which recommendations must happen first
-

Session 5 — Research Lab and Advanced Tools (60–90 minutes)

19. Open the Research Lab (</research-lab>)

20. Hospital Financial Scorecard (Section 4)

- Enter Vermont-approximate parameters: 40% Medicaid, 35% commercial, 25% Medicare payer mix
- Apply a -5% Medicaid rate change (modeling a rate cut scenario)
- See the operating margin impact and compare to the Rural PPS peer benchmark

21. Policy Simulator (Section 4)

- Open the Global Budget modeler
- Set the all-payer growth rate cap at 3.5% (Vermont's current target)
- Compare to a scenario at 2.5% (a more aggressive target)
- Observe how tightening the growth cap affects projected hospital financial positions

22. **Workforce Modeler** (Section 6)
 - Set specialty to Primary Care
 - Enter Vermont-approximate data: current supply, rural location, modest demand growth
 - Run the 10-year supply-demand projection
 - Apply the NHSC loan repayment program — see the projected impact on the supply gap
23. **Evidence Library** (Section 6)
 - Search for "global budget" in the CMMI model tracker
 - Review the Maryland All-Payer Model evaluation — note outcomes and lessons for Vermont
24. **Ask the AI Analyst a synthesis question**
 - Ask: *"Based on the Research Lab analysis I've done today — hospital financial modeling, workforce supply, and evidence from Maryland — what are the three most important operational prerequisites Vermont must have in place before tightening the AHEAD global budget?"*
-

Session 6 — Synthesis and Feedback (30 minutes)

25. **Review your bookmarked articles** (navigate to [/account](#) → Bookmarks)
26. **Download your AI Analyst transcript** from [/chat](#) (click the download button in the top right of the chat interface)
27. **Share your observations** with the HTR team — contact information provided separately with your access code

16. Frequently Asked Questions

- Q: What does "MVP · FOR TESTING ONLY" mean? Is the platform incomplete?** A: MVP stands for Minimum Viable Product — the first complete, functional version of the platform that is ready for external review. All analytical frameworks, Vermont-specific tools, and content are fully operational. The watermark is a signal that you are one of the first people to access the system, not that the platform is broken or unfinished. It will be removed when the platform opens to the public.
- Q: The faculty names on the Academy page show "Faculty #1," "Faculty #2," etc. Why?** A: Faculty names are anonymized during the early-access period to protect the privacy of instructors before the platform's public launch. Role, bio, and course content remain fully visible.
- Q: Some advisory content mentions "Competitor #1," "Competitor #2," "Competitor #3." What does that mean?** A: References to real competitor firm names have been temporarily replaced with placeholders for the early-access period for legal and strategic reasons. This affects only a small number of advisory positioning statements and does not affect any analytical content.
- Q: Is the data in the Act 167 Simulator real or synthetic?** A: The simulator uses a combination of real data (Oliver Wyman report figures, publicly available GMCB hospital financial data, Vermont Census/ACS demographic data) and synthetic projections where actual data has not yet been fully ingested. A persistent "Beta — Synthetic Data" badge on the simulator header serves as a constant reminder. All figures are illustrative for scenario-planning purposes.
- Q: Can I share what I see in the platform?** A: During the early-access period, please treat platform content as confidential. Do not share your access code with others. Do not publish screenshots or reports based on platform content without prior authorization from the HTR team. This restriction will be lifted at public launch.
- Q: My AI Analyst conversation disappeared. Where did it go?** A: AI Analyst conversations are stored only in your browser's local storage — not on HTR's servers. If you clear your browser cache or switch browsers, the conversation history is lost. Use the **Download Transcript** button in the full chat interface ([/chat](#)) to save important conversations before clearing your browser.

Q: Who built this platform? A: HTR is an independent health transformation research and advisory institution. It has no vendor relationships, no political affiliations, and no financial stake in the outcome of Vermont's healthcare reform decisions. The platform was built to serve decision-makers who need an evidence-first, complete analytical foundation — not a platform with a stake in which reforms are adopted.

Q: How current is the data? A: Most federal data sources have 12–18 month reporting lags. HTR applies a currency adjustment methodology that weights more recent data more heavily. The HTI is updated quarterly. State Performance Index scores are updated semi-annually. Vermont-specific content is reviewed and updated by HTR's Vermont advisory team on a rolling basis. Data vintage is noted wherever relevant.

Q: What happens if I find an error or a piece of content that seems wrong? A: This is exactly what the early-access period is for. Please contact the HTR team with specific notes — page URL, what you saw, and why you believe it may be incorrect. Your feedback directly shapes the final platform before public launch.

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